



Primary Medical Staffing, inc.

To: Ralph

Fax: 888-228-3798

Fax: 844-215-4265

Phone: 305-819-2343

Phone:

Date: 12/19/2022

Re: Addison Strachan

Pages: 9 pages including Cover.

☒ **Urgent**

☒ **For Review**

☐ **Please Comment**

☐ **Please Reply**

☐ **Please Recycle**

Comments:

Good morning!

The updated orders for patient Addison Strachan from her recent hospital stay. If you have any questions, please feel free to contact us at (305) 819-2343.

Thank you in advance,

Primary Medical Staffing, Inc.

Nicklaus Children's Hospital
3100 SW 62 Avenue
Miami, FL 33155-3009
Phone: 305-666-6511
Ordering MD Phone: 305-666-6511

Patient Information

MRN: 1117260534

Name: **STRACHAN, ADDISON**
Location: **PICUT**
Room: **3023 A2**
Date of Birth: **07/28/17**
Age: **5 Years**
Sex: **Female**
Height:
Weight:

FIN: **1222865829**
Admit Date: **12/07/22**
Attending MD: **Ladd MD , Hugh J**
Ordering Provider: **Lopez APRN , Kristen M**
LOS: **12 Days**
Attending NPI#: **1831356906**
Attending Lic#: **ME106859**
Ordering Provider NPI#: **1245616861**

Allergies: **acetaminophen, simethicone, Mylicon, Mylanta, Cows Milk Protein**

Ordering Information

Order Action: Order

Order: DME - MISC

Special Instructions: **resume home health as previously ordered**

Diagnoses :
(Included)

Order ID: 2097912891

Comments:

Ordering MD: Lopez APRN , Kristen M
Electronically signed

Entered By: Lopez APRN , Kristen M
Order Date/Time: **12/17/22 EST**
Communication Type: **Written**

Discharge Summary

STRACHAN, ADDISON - 1117260534

* Final Report *

*** Final Report *****Document Has Been Revised****Discharge Summary Note**Patient: **STRACHAN, ADDISON** MRN: 1117260534 FIN: 1222865829

Age: 5 years Sex: Female DOB: 07/28/2017

Associated Diagnoses: **Precocious puberty; Diabetes insipidus; Adrenal insufficiency; Acute respiratory failure with hypoxemia; MRSA pneumonia; Gastrostomy status**Author: **Ceruto APRN , Mara M****Discharge Information****Discharge Summary Information:** 12/7/2022, Discharged 12/18/2022.

Admitting Diagnosis: Precocious puberty (ICD10-CM E30.1, Discharge, Medical), Diabetes insipidus (ICD10-CM E23.2, Discharge, Medical), Adrenal insufficiency (ICD10-CM E27.40, Discharge, Medical), Acute respiratory failure with hypoxemia (ICD10-CM J96.01, Discharge, Medical), MRSA pneumonia (ICD10-CM J15.212, Discharge, Medical), Gastrostomy status (ICD10-CM Z93.1, Discharge, Medical).

Discharge physician: Martinez MD , Paul A.

Discharge diagnosis: Precocious puberty (ICD10-CM E30.1, Discharge, Medical), Diabetes insipidus (ICD10-CM E23.2, Discharge, Medical), Adrenal insufficiency (ICD10-CM E27.40, Discharge, Medical), Acute respiratory failure with hypoxemia (ICD10-CM J96.01, Discharge, Medical), MRSA pneumonia (ICD10-CM J15.212, Discharge, Medical), Gastrostomy status (ICD10-CM Z93.1, Discharge, Medical).

Hospital Course**Hospital Course**

Addison is a 5 year old female with complex medical history including panhypopituitarism, spastic quadriplegia, global developmental delay, G-tube dependence, intermittent home O2 use admitted in acute hypoxemic respiratory failure. Per mother, she has been at baseline state of health until yesterday when she appeared irritable and fussy and started having desats into the low 80's. Normally she is on RA, but was placed 3L NC with some improvement. Over the last 24 hrs, she continued to worsen and have further episodes of desaturations. Out of concern, she gave a dose of stress steroids and brought to NCH ED for evaluation. Of note, Addison was admitted to NCH PICU (acute respiratory failure in setting of aspiration pneumonia in November 2022. She was discharged home on ciprofloxacin and amoxicillin which she completed 11/28/22.

NCH ED O2% was in the low 70s for which she was intubated uneventfully and placed on invasive mechanical ventilation with improvement in saturations. Lab work revealed WBC 14k with bandemia of 24%. CRP 1.8. Procalcitonin 0.5. Patient was given a dose of Zosyn and vancomycin as well as another dose of stress dose steroids.

PICU Course (12/7-12/18): Arrived to PICU intubated and mechanically ventilated, started on continuous sedation. Extubated 12/11 to NIV, weaned to CPAP 12/15, HFNC 12/16 and now breathing comfortably on Nasal cannula. Failed Room air trial. Due to RLL atelectasis, pulmonary toileting regimen adjusted per Pulmonology, including albuterol and oscillating lung expansion therapy/ CPT every 4 hrs and fluticasone BID. Pending Polysomnogram, recommend obtaining study outpatient to determine CPAP settings. S/p Clindamycin for MRSA tracheitis. Blood culture positive for Staphylococcus Capitis, likely contaminant; repeat Blood culture negative. S/p fentanyl and

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dexmedetomidine drip, due to agitation upon discontinuation of sedation started on Clonidine. Addison is awake, tolerating sitting out of bed in chair without desats on nasal cannula, tolerating g-tube feeds at goal and at baseline activity level. Given improvement in clinical status plan to discharge home pending follow-up with pediatrician in 48 hrs and follow-up with Pulmonology this week. Plan to wean clonidine every other day, provided with written wean schedule. Continuing new pulmonary toileting regimen established inpatient by Pulmonology and home desmopressin and hydrocortisone regimen per Endocrinology recommendations. Discussed with Mother signs and symptoms that warrant immediate medical attention, in addition to the importance of obtaining new Polysomnogram. Until sleep study can be obtained, recommend CPAP 7 at home that has been tolerated inpatient without desats. Mother in agreement with plan of care, all questions and concerns addressed at this time..

Procedure

Procedures Performed during Hospital Stay?

No surgeries or procedures were performed during this visit.

Physical Examination

Pertinent Positives on Discharge Physical Exam: Temperature Axillary 97.0 - 97.5 DegF

Heart Rate Monitored 69 - 116 bpm
 Respiratory Rate 16 - 43 br/min
 Systolic Blood Pressure 80 - 99 mmHg
 Diastolic Blood Pressure 50 - 71 mmHg
 Mean Arterial Pressure, Cuff 61 - 81 mmHg
 SpO2 92 - 100 %

General: No acute distress.

Eye: Pupils are equal, round and reactive to light, Normal conjunctiva, No scleral icterus .

HENT: Normocephalic, Oral mucosa is moist.

Neck: Supple, Non-tender, No lymphadenopathy.

Respiratory: Lungs are clear to auscultation, Respirations are non-labored, Breath sounds are equal, Symmetrical chest wall expansion, Upper airway transmitted sounds. Good bilateral air entry. Nasal cannula

Cardiovascular: Normal rate, Regular rhythm, No murmur, Normal peripheral perfusion.

Gastrointestinal: Soft, Non-tender, Non-distended, Normal bowel sounds, No organomegaly, G tube in place, site c/d/i.

Integumentary: Warm, Dry, Pink.

Musculoskeletal: No swelling, Scoliosis .

Neurologic: Awake, GDD. .

Results Review

Laboratory results

Lab results

12/18/2022 10:46 EST	NA iSTAT	138 mmol/L
12/18/2022 09:24 EST	NA iSTAT	139 mmol/L
	Glucose iSTAT	115 mg/dL
	Ionized Calcium iSTAT	1.20 mmol/L
	HGB iSTAT	12.9 g/dL
	HCT iSTAT	38 % PCV
	Sample Cap	CAPILLARY
	Site Cap	.
	FiO2 Cap	0 % NA

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	RRIV Cap	0 bpm NA
	PEEP Cap	0.00 NA
	PIP Cap	0.00 NA
	BP Cap	0.0 mmHg NA
	Temp Cap	37.0 DegC NA
	pH Cap	7.45 HI
	pCO2 Cap	36.1 mmHg
	pO2 Cap	71.0 mmHg LOW
	BE Cap	1.0 mmol/L
	HCO3 Cap	25.2 mmol/L
	CO2 Totl Cap	26.0 mmol/L
	O2 Sat Cap	95.00 %
12/18/2022 03:24 EST	Sodium Lvl	139 mmol/L
	Potassium Lvl	4.8 mmol/L
	Chloride	108 mmol/L
	CO2	21.4 mmol/L
	Glucose Lvl	104 mg/dL
	BUN	5 mg/dL LOW
	Creatinine	0.31 mg/dL
	BUN/Creat Ratio	16.13 NA
12/17/2022 20:58 EST	Calcium Lvl	9.8 mg/dL
	NA iSTAT	140 mmol/L
	K iSTAT	6.4 mmol/L HI
	Glucose iSTAT	91 mg/dL
	Ionized Calcium iSTAT	1.19 mmol/L
	HGB iSTAT	13.6 g/dL
	HCT iSTAT	40 % PCV
	Sample Cap	CAPILLARY
	Site Cap	-
	FIO2 Cap	0 % NA
	RRIV Cap	0 bpm NA
	PEEP Cap	0.00 NA
	PIP Cap	0.00 NA
	BP Cap	0.0 mmHg NA
	Temp Cap	37.0 DegC NA
	pH Cap	7.56 HI
	pCO2 Cap	28.2 mmHg LOW
	pO2 Cap	59.0 mmHg LOW
	BE Cap	3.0 mmol/L HI
	HCO3 Cap	25.3 mmol/L
	CO2 Totl Cap	26.0 mmol/L
	O2 Sat Cap	94.00 %
12/17/2022 18:56 EST	NA iSTAT	136 mmol/L
	K iSTAT	5.9 mmol/L HI
	Glucose iSTAT	127 mg/dL HI
	Ionized Calcium iSTAT	1.16 mmol/L
	HGB iSTAT	12.9 g/dL
	HCT iSTAT	38 % PCV

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	Sample Cap	CAPILLARY
	Site Cap	-
	FiO2 Cap	0 % NA
	RRIV Cap	0 bpm NA
	PEEP Cap	0.00 NA
	PIP Cap	0.00 NA
	BP Cap	0.0 mmHg NA
	Temp Cap	37.0 DegC NA
	pH Cap	7.43 HI
	pCO2 Cap	39.1 mmHg
	pO2 Cap	73.0 mmHg LOW
	BE Cap	2.0 mmol/L
	HCO3 Cap	25.9 mmol/L
	CO2 Totl Cap	27.0 mmol/L
	O2 Sat Cap	95.00 %
12/17/2022 06:16 EST	NA iSTAT	145 mmol/L HI
	K iSTAT	3.7 mmol/L
	Glucose iSTAT	107 mg/dL
	Ionized Calcium iSTAT	1.07 mmol/L LOW
	HGB iSTAT	10.2 g/dL LOW
	HCT iSTAT	30 % PCV LOW
	Sample Ven	VENOUS
	Site Ven	-
	FiO2 Ven	0 % NA
	RR/IVM Ven	. bpm
	PEEP Ven	0.00 NA
	PIP Ven	0.00 NA
	BP Ven	0.0 mmHg NA
	Temp Ven	37.0 DegC NA
	pH Ven	7.49 HI
	pCO2 Ven	28.3 mmHg LOW
	pO2 Ven	49.0 mmHg NA
	BE Ven	-2.0 mmol/L
	HCO3 Ven	21.7 mmol/L LOW
	CO2 Totl Ven	23.0 mmol/L LOW
	O2 Sat Ven	88.0 % LOW
12/17/2022 05:28 EST	Sodium Lvl	142 mmol/L
	Potassium Lvl	3.6 mmol/L
	Chloride	106 mmol/L
	CO2	27.4 mmol/L
	Glucose Lvl	123 mg/dL HI
	BUN	5 mg/dL LOW
	Creatinine	0.33 mg/dL
	BUN/Creat Ratio	15.15 NA
	Calcium Lvl	9.3 mg/dL

Radiology results

Discharge Summary

STRACHAN, ADDISON - 1117260534

* Final Report *

*** Final Report ***

Reason For Exam

resp distress;Respiratory Distress

POWERSCRIBE REPORT

History: Cerebral palsy and aspiration pneumonia

Frontal chest study from 6:12 AM is compare with 12/14/2022.

CHEST: Improved aeration of the right lung is seen. Mild residual right central infiltrate/atelectasis seen. Small right pleural effusion versus residual pleural thickening identified. Left lung is well aerated. Heart size is normal. Residual scoliosis with spine hardware in place noted.

Signature Line

***** Final *****

Dictated by: Medina MD , L Santiago

Dictated DT/TM: 12/16/2022 9:13 am

Transcribed By: Medina MD , L Santiago

Transcribed DT/TM: 12/16/2022 9:11

Signed by: Medina MD , L Santiago

Signed (Electronic Signature): 12/16/2022 9:13 am

Thank you for the opportunity to participate in the health care of this patient.

I have personally reviewed the images and the report.

This document has an image

Note type: XR Portable Chest 1 View

Result date: December 16 2022 06:34 EST

Status: Auth (Verified)

Note title: XR Portable Chest 1 View

Initiated by: Mendiluzza , Jenni on December 16 2022 06:34 EST

Final signature: Medina MD , L Santiago on December 16 2022 09:13 EST

Encounter info: 1222865829, Nicklaus Children's Hospital, Inpatient, 12/07/2022 -

Discharge Plan

Discharge Summary Plan

Discharge Status: improved.

Discharge disposition: discharge to home (into the care of family member, with home health care, Respiratory therapy).

Prescriptions: prescribed medications (Selected)

Prescriptions

Prescribed

cloNIDine 0.1mg/1mL Oral Solution: 0.1 mg, GTUBE, q8hr, for 7 day(s), 25 mL, 0 Refill(s)

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diazePAM 5 mg/5 mL oral solution: 2 mL, Oral, q8hr, for 30 day(s), 180 mL, 5 Refill(s)
fluticasone CFC free 110 mcg/inh inhalation aerosol with adapter: 2 puff(s), INH, BID, for 30 day(s),
obtain refill from PCP or pulmonology, 1 EA, 1 Refill(s)

Education and Follow-up

Discharge Instructions/Counseling given to caregiver: yes.

Discharge Planning: Patient Education provided Chronic Lung Disease: Avoiding Irritants and Allergens, Preventing Pneumonia, Preventing Common Respiratory Infections, Follow-up with Maria Franco Within 1 week Call for followup appointment; Nicole Torres Within 1 to 2 days Call for followup appointment.

Signature Line

[Electronically Signed on: 12/18/2022 01:52 PM EST]

Ceruto APRN , Mara M

[Electronically Signed on: 12/18/2022 02:05 PM EST]

Ceruto APRN , Mara M

Martinez MD , Paul A

[Verified on: 12/18/2022 01:52 PM EST]

Ceruto APRN , Mara M

Note type: Discharge Summary
Result date: December 18 2022 11:13 EST
Status: Modified
Note title: Discharge Summary Note
Initiated by: Ceruto APRN , Mara M on December 18 2022 11:20 EST
Final signature: Ceruto APRN , Mara M on December 18 2022 13:52 EST
Encounter info: 1222865829, Nicklaus Children's Hospital, Inpatient, 12/07/2022 -

Strachan, Addison DOB 07/28/2017**Clonidine wean schedule**

Sunday 12/18/22	Monday 12/19/22	Tuesday 12/20/22	Wednesday 12/21/22
Clonidine 1 ml 3x day	Clonidine 0.5 ml 3x day	Clonidine 0.5 ml 3x day	Clonidine 0.5 ml 2x day

Thursday 12/22/22	Friday 12/23/22	Saturday 12/24/22
Clonidine 0.5 ml 2x day	Clonidine 0.5 ml Daily	Clonidine 0.5 ml Daily